MIKE O'CALLAGHAN MILITARY MEDICAL CENTER

4700 N Las Vegas Blvd
Nellis AFB, NV 89191

99th MDG Nellis AFB

TO:	ACTIVE HOME HEALTH
	Re: Downs, Jimmy
PHONE:	775-471-9815
FAX:	702-726-9551
FROM:	Zorana Alu-Acol, BSN, RN, CCM, CCP Inpatient Clinical Nurse Case Manager - Discharge Planner
COMPANY:	Mike O'Callaghan Federal Medical Center
PHONE:	Comm: (702) 679-9691 Work Cell: (702) 461-5444
FAX:	(702) 653-3070
Date:	4 / 13 /2026

TOTAL PAGES: ____ (Including Cover) or Multiple pages

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COMMENTS:

DC: 4/13/2026

HH RN and PT

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MOFH Form O-184

34/1

Registration Form

Patient Information

Patient: Downs, Jimmy
Address: 4361 CHERRYSTONE CT
LAS VEGAS, NV 891215128
Date of Birth: 01/30/1951
Sex: Male
Home: 7024334825
Mobile:
Work:

Guarantor Information

Name: Downs, Jimmy
Address: 4361 CHERRYSTONE CT
LAS VEGAS, NV 891215128
Date of Birth: 01/30/1951
Sex: Male
Home: 7024334825
Mobile:
Work:

Related Persons

Role	Name	Relationship	Home	Mobile
Emergency Contact	Downs, Anna	NK/EC - Wif...	(702) 438-4166	(702) 438-4166
Next of Kin	Downs, Anna	NK/EC - Hus...	(702) 438-4166	(702) 438-4166

Primary Insurance

Payer Name: TRICARE
HP Name: TRICARE PLUS
Contact: -
Group Nbr: 346
Member Nbr: 00226093700
Name on Card:
Start Date: 07/17/2025

Medicare A/B
6MC1 XØ1 VE33

Primary Subscriber Information

Name: Downs, Jimmy
Reltn: Self
Address: 4361 CHERRYSTONE CT
LAS VEGAS, NV 891215128
Date of Birth: 01/30/1951
Home: 7024334825
Employer: U.S. Marine Corps

Secondary Insurance

SSN: 250-88-7038

Payer Name: TRICARE
HP Name: 602 Direct Care and TRICARE Mail Order and Retail ...
Contact: - 8888888888
Group Nbr: 602
Member Nbr: 00226093700
Name on Card:
Start Date: 07/17/2025

Secondary Subscriber Information

Name: Downs, Jimmy
Reltn: Self
Address: 4361 CHERRYSTONE CT
LAS VEGAS, NV 891215128
Date of Birth: 01/30/1951
Home: 7024334825
Employer: U.S. Marine Corps

Assignment of Benefits

I authorize direct remittance of payment of all insurance benefits, including Medicare, if I am a Medicare beneficiary, to this provider for all covered medical services and supplies provided to me. This assignment will remain in effect until revoked by me in writing. A photocopy of this assignment is to be considered as valid as an original.

Authorization to Release Information

I authorize the release of any medical or any other information to the Centers for Medicare and Medicaid (CMS), my insurance carrier(s), or other entity necessary to determine insurance benefits or the benefits payable for related medical services and/or supplies provided to me by this provider. A copy of this authorization will be sent to CMS, my insurance carrier(s), or other medical entity, if requested. This authorization will remain in effect until revoked by me in writing. A photocopy of this assignment is to be considered as valid as an original.

Signature of Patient or Legal Guardian **Date**

Print Name of Patient or Legal Guardian

MIKE O'CALLAGHAN
MILITARY MEDICAL CENTER
4700 N Las Vegas Blvd
Nellis AFB, NV 89191

(This form is subject to the Privacy Act of 1974
Use Blanket PAS - DD Form 2005)

AF IMT 781, 19880601, V2
Previous Edition will be used.

MULTIPLE ITEM PRESCRIPTION

Rx	Directions
1. Home Health PT	Strengthening, balance & transfers, gait & mobility, & Home safety assessment.
2. Home Health RN	Initiate home evaluation and treat Disease management and medication reconciliation.
3.	

a. Full Name of Patient (AGE if under 12) (Use Plastic Card or PRINT)
Downs, Jimmy
DOB: 01/30/1951
SSN: 250-88-7038

Prescriber Signature

Prescriber Name: Nicholas Symons, MD
NPI: 1154459120

Prescriber's Phone: (702) 653-3800
Prescriber's Fax: (702) 653-3070

b. SSN of Sponsor

c. Patient's Address (Mandatory for Controlled Substances)
4361 Chemystone Ct.
Las Vegas, NV 89121

d. Work/Home Telephone (For emergency only)

Date: 13 APR 2026

Drugs listed in Schedule II must always be separate from the drugs in Schedules III, IV, and V. Therefore, do not write noncontrolled and controlled drugs on the same AF Form 781, and do not combine drugs listed in Schedule II on the same prescription with drugs listed in Schedules III, IV, and V.

Medication List

Patient : Downs, Jimmy
Address : 4361 CHERRYSTONE CT
LAS VEGAS, NV 891215128
Phone : + 7024334825

Med Rec # : 3728595000001
DOB : 01/30/51
Sex : Male
Physician : ALEDIGNE, YEABSRA K, MD, Internal Medicine

All Active Medications

Medications

acetaminophen
975 mg, Oral, Tablet, TID, PRN pain, mild (DVPRS, 1-4), First Dose: 04/12/2026 00:16:00 PDT

acetaminophen (Tylenol Extra Strength 500 mg oral tablet)
2 tab(s), Oral, TID, PRN pain or fever, # 200 tab(s), 1 total refill(s), Maintenance, 2 tab(s) Oral TID,PRN:as needed for pain or fever, Pharmacy: DOD NELLIS PHARMACY [Not filled]

apixaban
5 mg, Oral, Tablet, BID, First Dose: 04/12/2026 10:00:00 PDT, 04/12/2026 00:16:00 PDT

apixaban (apixaban 5 mg oral tablet)
1 tab(s), Oral, BID, # 180 tab(s), 3 total refill(s), Maintenance, Pharmacy: DOD NELLIS PHARMACY [Federal Rx: #180 last filled 02/03/26]

bumetanide (bumetanide 2 mg oral tablet)
1 tab(s), Oral, Daily, # 30 tab(s), 0 total refill(s), Maintenance

cefTRIAXone
1 g, IV Piggyback (Intravenous), Injection, every 24 hr, Indication: Urinary, complicated UTI/pyelonephritis, Administer over: 30 minutes, Order Duration: 3 days, First Dose: 04/12/2026 22:00:00 PDT, Stop Date: 04/15/2026 21:59:00 PDT

dextrose 50% in water (Dextrose 50% Sliding Scale)
25-50mL, IV Push (Intravenous), Form: Injection, As Directed PRN hypoglycemia (see comment), First Dose: 04/12/2026 00:09:00 PDT

erythromycin ophthalmic (erythromycin 0.5% eye ointment [3.5g])
See Instructions, # 3.5 g, 3 total refill(s), Soft Stop [Federal Rx: #3.5 last filled 03/13/25]

finasteride
5 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT, Hazardous, 04/12/2026 00:16:00 PDT

finasteride (finasteride 5 mg oral tablet)
1 tab(s), Oral, Daily, # 90 tab(s), 3 total refill(s), Maintenance, Pharmacy: DOD NELLIS PHARMACY [Federal Rx: #90 last filled 03/30/26]

folic acid
1 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT, 04/12/2026 00:26:00 PDT

glucagon
1 mg, IntraMuscular, Injection, every 15 min, PRN other (see comment), Order Duration: 2 dose(s), First Dose: 04/12/2026 00:09:00 PDT, Stop Date: Limited # of times, 04/12/2026 00:09:00 PDT

glucose
15 g, Oral, Gel-Oral, As Directed, PRN hypoglycemia (see comment), First Dose: 04/12/2026 00:09:00 PDT, 04/12/2026 00:09:00 PDT

insulin aspart
5 unit(s), SubCutaneous, Injection-Insulin, AC (before meals), First Dose: 04/12/2026 00:09:00 PDT, 04/12/2026 00:09:00 PDT

insulin glargine (insulin glargine 100 units/mL subcutaneous solution)
15 unit(s), SubCutaneous, Injection-Insulin, every day at bedtime, First Dose: 04/12/2026 00:09:00 PDT, 04/12/2026 00:09:00 PDT

lidocaine topical (Lidoderm 5% topical film)
1 patch(es), Topical, Daily, Leave on for up to 12 hours within a 24 hour period (12 hours on, 12 hours off), # 30 patch(es), 3 total refill(s), Maintenance, 1 patch(es) Topical Daily,Instr:Leave on for up to 12 hours within a 24 hour period (12 hours...

lidocaine topical (Lidoderm 5% topical patch)
1 patch(es), Topical, Patch, Daily, First Dose: 04/12/2026 09:00:00 PDT

Medication List

Patient : Downs, Jimmy
Address : 4361 CHERRYSTONE CT
LAS VEGAS, NV 891215128
Phone : - 7024334825

Med Rec # : 3728595000001
DOB : 01/30/51
Sex : Male
Physician : ALELIGNE, YEABSRA K, MD, Internal Medicine

All Active Medications continued...

Medications continued...

metoprolol (Toprol-XL 50 mg oral tablet, extended release)
1 tab(s), Oral, Daily, for blood pressure, # 90 tab(s), 3 total refill(s), Maintenance, Pharmacy: DOD NELLIS PHARMACY [Federal Rx: #90 last filled 03/30/26]

metoprolol (Toprol-XL)
50 mg, Oral, Tablet-Extended Release, Daily, First Dose: 04/12/2026 09:00:00 PDT, 04/12/2026 00:16:00 PDT

nicotine (nicotine 7 mg/24 hr transdermal film, extended release)
1 patch(es), TransDermal, Patch, Daily, First Dose: 04/12/2026 09:00:00 PDT, 04/12/2026 00:27:00 PDT

omeprazole
40 mg, Oral, Capsule-Delayed Release, Daily, First Dose: 04/12/2026 07:30:00 PDT

omeprazole (omeprazole 40 mg oral delayed release capsule)
1 cap(s), Oral, Daily, 30 to 60 minutes before a meal, # 90 cap(s), 3 total refill(s), Maintenance, Pharmacy: DOD NELLIS PHARMACY [Last filled 02/03/26]

ondansetron
4 mg, Oral, Tablet-Disintegrating, every 6 hr, PRN nausea, First Dose: 04/12/2026 00:26:00 PDT, 04/12/2026 00:28:00 PDT

polyethylene glycol 3350 (MiraLax)
17 g, Oral, Powder-Oral-Gram, BID, First Dose: 04/12/2026 10:00:00 PDT, 04/12/2026 01:09:00 PDT

potassium chloride (potassium chloride 20 mEq oral tablet, extended release)
1 tab(s), Oral, Daily, # 90 tab(s), 3 total refill(s), Maintenance, Pharmacy: DOD NELLIS PHARMACY [Federal Rx: #90 last filled 02/03/26]

rosuvastatin (Crestor 20 mg oral tablet)
1 tab(s), Oral, Daily, for cholesterol, # 90 tab(s), 3 total refill(s), Maintenance, Pharmacy: DOD NELLIS PHARMACY [Federal Rx: #90 last filled 02/03/26]

rosuvastatin (Crestor)
10 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT

Flowsheet Print Request

Patient: Downs, Jimmy
MRN: 3728595000001

Printed by: ALU-ACOL, ZORANA P, RN
Date Range (Admission - Current): 04/11/2026 21:10 PDT - 04/13/2026 12:38 |Printed on: 04/13/2026 12:38 PDT

Event Date	Event	Result	Ref. Range	Status
04/13/2026 11:58 PDT	POC Glucose	273 (H)	(74 - 106)	
04/13/2026 11:05 PDT	WBC	8.94	(4.23 - 9.07)	
	RBC	5.39	(4.63 - 6.08)	
	Hemoglobin	15.4	(13.7 - 17.5)	
	Hematocrit	45.9	(40.1 - 51.0)	
	MCV	85.2	(79.0 - 92.2)	
	MCH	28.6	(25.7 - 32.2)	
	MCHC	33.6	(32.3 - 36.5)	
	RDW	15.7 (H)	(11.6 - 14.4)	
	Platelets	226	(163 - 337)	
	MPV	11.2	(9.4 - 12.4)	
	Differential?	Not Indicated		
	Neutrophil % Auto	77.1 (H)	(34.0 - 67.9)	
	Lymphocyte % Auto	15.1 (L)	(21.8 - 53.1)	
	Monocyte % Auto	5.3	(5.3 - 12.2)	
	Eosinophil % Auto	0.9	(0.8 - 7.0)	
	Basophil % Auto	0.4	(0.2 - 1.2)	
	Imm. Granulocyte %	1.20 (H)	(0.11 - 0.32)	
	Neutro Absolute	6.89 (H)	(1.78 - 5.38)	
	Lymph Absolute	1.35	(1.32 - 3.57)	
	Mono Absolute	0.47	(0.30 - 0.82)	
	Eos Absolute	0.08	(0.04 - 0.54)	
	Baso Absolute	0.04	(0.01 - 0.08)	
	nRBC % Auto	0.0	(0.0 - 0.2)	
	nRBC Absolute	0.000	(0.000 - 0.012)	
	Imm. Granulocyte Absolute	0.1		
	Sodium	136	(136 - 145)	
	Potassium Lvl	4.2	(3.5 - 5.1)	
	Chloride	104	(98 - 107)	
	CO2	23.6	(20.0 - 31.0)	
	AGAP	8	(3 - 16)	
	BUN	27 (H)	(9 - 23)	
	Creatinine Level	2.18 (H)	(0.70 - 1.30)	
	eGFR CKD EPI	31 *		
	Glucose Lvl	261 (H)	(74 - 106)	
	Calcium	9.1	(8.7 - 10.4)	
	Albumin	3.4	(3.2 - 4.8)	
	Phosphorus	3.3	(2.4 - 5.1)	
04/13/2026 09:50 PDT	WBC	NT * (c)	(4.23 - 9.07)	
	RBC	NT * (c)	(4.63 - 6.08)	
	Hemoglobin	NT * (c)	(13.7 - 17.5)	
	Hematocrit	NT * (c)	(40.1 - 51.0)	
	MCV	NT * (c)	(79.0 - 92.2)	
	MCH	NT * (c)	(25.7 - 32.2)	
	MCHC	NT * (c)	(32.3 - 36.5)	
	RDW	NT * (c)	(11.6 - 14.4)	
	Platelets	NT * (c)	(163 - 337)	
	MPV	NT * (c)	(9.4 - 12.4)	
	Differential?	NT * (c)		

Flowsheet Print Request

Patient: Downs, Jimmy
MRN: 3728595000001

Printed by: ALU-ACOL, ZORANA P, RN
Date Range (Admission - Current): 04/11/2026 21:10 PDT - 04/13/2026 12:38 |Printed on: 04/13/2026 12:36 PDT

Event Date	Event	Result	Ref. Range	Status
	Neutrophil % Auto	NT * (c)	(34.0 - 67.9)	
	Lymphocyte % Auto	NT * (c)	(21.8 - 53.1)	
	Monocyte % Auto	NT * (c)	(5.3 - 12.2)	
	Eosinophil % Auto	NT * (c)	(0.8 - 7.0)	
	Basophil % Auto	NT * (c)	(0.2 - 1.2)	
	Imm. Granulocyte %	NT * (c)	(0.11 - 0.32)	
	Neutro Absolute	NT * (c)	(1.78 - 5.38)	
	Lymph Absolute	NT * (c)	(1.32 - 3.57)	
	Mono Absolute	NT * (c)	(0.30 - 0.82)	
	Eos Absolute	NT * (c)	(0.04 - 0.54)	
	Baso Absolute	NT * (c)	(0.01 - 0.08)	
	nRBC % Auto	NT * (c)	(0.0 - 0.2)	
	nRBC Absolute	NT * (c)	(0.000 - 0.012)	
	Imm. Granulocyte Absolute	NT * (c)		
	Sodium	NT * (c)	(136 - 145)	
	Potassium Lvl	NT * (c)	(3.5 - 5.1)	
	Chloride	NT * (c)	(98 - 107)	
	CO2	NT * (c)	(20.0 - 31.0)	
	ACAP	NT * (c)	(3 - 16)	
	BUN	NT * (c)	(9 - 23)	
	Creatinine Level	NT * (c)	(0.70 - 1.30)	
	eGFR CKD EPI	NT * (c)		
	Glucose Lvl	NT * (c)	(74 - 106)	
	Calcium	NT * (c)	(8.7 - 10.4)	
	Albumin	NT * (c)	(3.2 - 4.8)	
	Phosphorus	NT * (c)	(2.4 - 5.1)	
04/13/2026 08:30 PDT	POC Glucose	156 (H)	(74 - 106)	
04/12/2026 20:48 PDT	POC Glucose	121 (H)	(74 - 106)	
04/12/2026 16:20 PDT	POC Glucose	209 (H)	(74 - 106)	
04/12/2026 11:20 PDT	POC Glucose	343 (H)	(74 - 106)	
04/12/2026 08:01 PDT	POC Glucose	290 (H)	(74 - 106)	
04/12/2026 05:53 PDT	WBC	8.80	(4.23 - 9.07)	
	RBC	4.75	(4.63 - 6.08)	
	Hemoglobin	13.6 (L)	(13.7 - 17.5)	
	Hematocrit	41.6	(40.1 - 51.0)	
	MCV	87.6	(79.0 - 92.2)	
	MCH	28.6	(25.7 - 32.2)	
	MCHC	32.7	(32.3 - 36.5)	
	RDW	15.6 (H)	(11.6 - 14.4)	
	Platelets	210	(163 - 337)	
	MPV	10.6	(9.4 - 12.4)	
	Differential?	Not Indicated		
	Neutrophil % Auto	71.0 (H)	(34.0 - 67.9)	
	Lymphocyte % Auto	19.9 (L)	(21.8 - 53.1)	
	Monocyte % Auto	6.9	(5.3 - 12.2)	
	Eosinophil % Auto	1.0	(0.8 - 7.0)	
	Basophil % Auto	0.3	(0.2 - 1.2)	
	Imm. Granulocyte %	0.90 (H)	(0.11 - 0.32)	
	Neutro Absolute	6.24 (H)	(1.78 - 5.38)	

Flowsheet Print Request

Patient: Downs, Jimmy
MRN: 3728595000001

Printed by: ALU-ACOL, ZORANA P, RN
Date Range (Admission - Current): 04/11/2026 21:10 PDT - 04/13/2026 12:38 Printed on: 04/13/2026 12:38 PDT

Event Date	Event	Result	Ref. Range	Status
	Lymph Absolute	1.75	(1.32 - 3.57)	
	Mono Absolute	0.61	(0.30 - 0.82)	
	Eos Absolute	0.09	(0.04 - 0.54)	
	Baso Absolute	0.03	(0.01 - 0.08)	
	nRBC % Auto	0.0	(0.0 - 0.2)	
	nRBC Absolute	0.000	(0.000 - 0.012)	
	Imm. Granulocyte Absolute	0.1		
	Sodium	136	(136 - 145)	
	Potassium Lvl	5.43	(3.5 - 5.1)	
	Chloride	105	(98 - 107)	
	CO2	25.4	(20.0 - 31.0)	
	AGAP	6	(3 - 16)	
	BUN	24 (H)	(9 - 23)	
	Creatinine Level	2.40 (H)	(0.70 - 1.30)	
	eGFR CKD EPI	27 *		
	Glucose Lvl	325 (H)	(74 - 106)	
	Calcium	8.5 (L)	(8.7 - 10.4)	
	Phosphorus	2.8	(2.4 - 5.1)	
	Magnesium Lvl	1.97	(1.6 - 2.6)	
	Hemoglobin A1c	11.2 * (H)	(- <=5.6)	
	eAvg Glucose	274 *		
	Cholesterol Total	84 *	(- <=199)	
	HDL Cholesterol	37.2 *	(40 - 60)	
	Triglycerides	82 *	(- <=149)	
	VLDL	16	(0 - 80)	
	LDL Direct	35 *	(0 - 99)	
04/12/2026 05:26 PDT	Ur Albumin	6.3		
	Ur Creat	79.7 *		
	Ur Albumin/Ur Creat Ratio	79		
04/11/2026 23:26 PDT	POC Glucose	324 (H)	(74 - 106)	
04/11/2026 23:25 PDT	POC Glucose	324 (H)	(74 - 106)	
04/11/2026 22:43 PDT	hsTroponin-I	32.37 *	(0.00 - 53.00)	
04/11/2026 22:13 PDT	UA Color	Yellow	(Yellow)	
	UA Clarity	Cloudy (A)	(Clear)	
	UA pH	8.5 (A)	(7.5)	
	UA Spec Gravity	1.029	(1.002 - 1.030)	
	UA Glucose	>=1000 (A)	(Negative)	
	UA Ketones	Negative	(Negative)	
	UA Blood	Trace (A)	(Negative)	
	UA Protein	30 (A)	(Negative)	
	UA Bili	Negative	(Negative)	
	UA Urobilinogen	1.0	(1.0)	
	UA Nitrite	Positive (A)	(Negative)	
	UA Leuk Esterase	Moderate (A)	(Negative)	
	UA Micro Ind?	Indicated		
	UA WBC	>100 (A)	(None Seen)	
	UA RBC	0-2 (A)	(None Seen)	
	UA Bacteria	4+		
	UA Epi Squam	0-5		

Flowsheet Print Request

Patient: Downs, Jimmy
MRN: 3728595000001

Printed by: ALU-ACOL, ZORANA P, RN
Date Range (Admission - Current): 04/11/2026 21:10 PDT - 04/13/2026 12:38 |Printed on: 04/13/2026 12:38 PDT

Event Date	Event	Result	Ref. Range	Status
	UA Hyaline Cast	0-2 (A)	(None Seen)	
	Urine Culture	In Progress See Report C		
04/11/2026 21:38 PDT	B-Type Natriuretic Peptide	245 * (H)	(- <=100)	
04/11/2026 21:35 PDT	WBC	8.68	(4.23 - 9.07)	
	RBC	4.90	(4.63 - 6.08)	
	Hemoglobin	13.8	(13.7 - 17.5)	
	Hematocrit	43.6	(40.1 - 51.0)	
	MCV	89.0	(79.0 - 92.2)	
	MCH	28.2	(25.7 - 32.2)	
	MCHC	31.7 (L)	(32.3 - 36.5)	
	RDW	15.3 (H)	(11.6 - 14.4)	
	Platelets	236	(163 - 337)	
	MPV	11.1	(9.4 - 12.4)	
	Neutrophil % Auto	74.2 (H)	(34.0 - 67.9)	
	Lymphocyte % Auto	16.7 (L)	(21.8 - 53.1)	
	Monocyte % Auto	7.5	(5.3 - 12.2)	
	Eosinophil % Auto	0.3 (L)	(0.8 - 7.0)	
	Basophil % Auto	0.3	(0.2 - 1.2)	
	Imm Granulocyte %	1.00 (H)	(0.11 - 0.92)	
	Neutro Absolute	6.43 (H)	(1.78 - 5.38)	
	Lymph Absolute	1.45	(1.32 - 3.57)	
	Mono Absolute	0.65	(0.30 - 0.82)	
	Eos Absolute	0.03 (L)	(0.04 - 0.54)	
	Baso Absolute	0.03	(0.01 - 0.08)	
	nRBC % Auto	0.0	(0.0 - 0.2)	
	nRBC Absolute	0.000	(0.000 - 0.012)	
	Imm Granulocyte Absolute	0.1		
	PT	11.5	(9.5 - 11.5)	
	INR	1.11 * (H)	(0.90 - 1.10)	
	Sodium	135 (L)	(136 - 145)	
	Potassium Lvl	5.4 (H)	(3.5 - 5.1)	
	Chloride	104	(98 - 107)	
	CO2	22.3	(20.0 - 31.0)	
	AGAP	9	(3 - 16)	
	BUN	24 (H)	(9 - 23)	
	Creatinine Level	2.79 (H)	(0.70 - 1.30)	
	eGFR CKD EPI	23 *		
	Glucose Lvl	556 * (H)	(74 - 106)	
	Calcium	9.1	(8.7 - 10.4)	
	Protein Total	6.3	(5.7 - 8.2)	
	Albumin	3.5	(3.2 - 4.8)	
	Phosphorus	2.9	(2.4 - 5.1)	
	Magnesium Lvl	2.3	(1.6 - 2.6)	
	Bilirubin Total	0.30	(0.20 - 1.10)	
	Bilirubin Direct	0.1	(- <=0.3)	
	Lipase Lvl	43	(12 - 53)	
	Alk Phos	75	(46 - 116)	
	ALT	9 (L)	(10 - 49)	
	AST	12	(- <=33)	

Flowsheet Print Request

Patient Downs, Jimmy
MRN: 3728595000001

Printed by: ALU-ACOL, ZORANA P, RN
Date Range (Admission - Current): 04/11/2026 21:10 PDT - 04/13/2026 12:38 Printed on: 04/13/2026 12:38 PDT

Event Date	Event	Result	Ref. Range	Status
	CK Total	101	(46 - 171)	
	Lactic Acid Plasma	1.88	(0.50 - 2.20)	
	hsTroponin-I	32.88 *	(0.00 - 53.00)	
	TSH, Sensitive	1.238 *	(0.500 - 5.000)	
	Ethanol Lvl	<3 *		
04/11/2026 21:33 PDT	POC pH (Ven)	7.35	(7.31 - 7.41)	
	POC pCO2 (Ven)	41	(41 - 51)	
	POC pO2 (Ven)	35 (L)		
	POC HCO3 (Ven)	22.7 (L)	(23.0 - 28.0)	
	POC sO2 (Ven)	65		
	POC BE (Ven)	-3 (L)	(-2 - 3)	
	POC TCO2 (Ven)	24	(24 - 29)	
	POC Calcium Ionized (Ven)	1.26	(1.12 - 1.32)	
	POC Hemoglobin calc (Ven)	15.0		
	POC Hematocrit (Ven)	44	(38 - 51)	
	POC Glucose (Ven)	547 * (L)	(70 - 105)	
	POC Potassium (Ven)	5.5 (H)	(3.5 - 4.9)	
	POC Sodium (Ven)	137 (L)	(138 - 146)	

0079A-Mike O'Callaghan Military Medical Center
4700 North Las Vegas Blvd
Building 1300
Nellis AFB, NV 89191-
(702) 653-3610

Patient Name:	**Downs, Jimmy**	Admit:	4/11/2026 21:10 PDT
MRN:	3728595000001	Discharge:	
FIN:	201841085	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
Sex/DOB/Age:	Male 1/30/1951 75 years	Location:	0079A-INP-MMMSU; 3134 MSU; 1
DOD ID (EDIPI):	1060909365	Veterans ID (ICN):	

Progress Notes

Document Type:	Progress Note
Service Date/Time:	4/13/2026 11:40 PDT
Result Status:	Auth (Verified)
Perform Information:	SYMONS, NICHOLAS P, MD, Internal Medicine (4/13/2026 11:45 PDT)
Sign Information:	SYMONS, NICHOLAS P, MD, Internal Medicine (4/13/2026 11:45 PDT)

Assessment/Plan

75 y/o PMHx of Afib/flutter on apixaban, NICM s/p ICD, HFrEF, CKDIII, HTN, HLD, h/o PE, morbid obesity, recurrent falls, and chronic weakness with baseline limited mobility requiring a walker admitted for failure to thrive, AKI, hyperkalemia, and hyperglycemia.

#Fall
#Weakness
Pt with extensive comorbidities and chronic weakness. Acute on chronic exacerbation likely multifactorial and includes UTI, hyperglycemia, dehydration/AKI, deconditioning. Pan scans were negative, low suspicion for fracture or stroke given his exam and work up. Pt with hx of chronic LE weakness worse on the left side. Low suspicion for syncope given his history. Potassium slightly elevated w/o ECG changes, hyperglycemic to the 500s initially but improved to the 300s with insulin and fluids. No other metabolic abnormalities. Will continue to monitor and treat any underlying metabolic/infectious cause, pend PT/OT eval.
- Vitals: q4hr
- Daily CBC, BMP, Mg, Phos
- Fall precautions
- PT evaluation
- Monitor electrolytes and glucose closely
- Treat any underlying infection or metabolic abnormalities

#Hyperglycemia
Glucose in the 500s initially, repeat in the low 300s after fluids an insulin in the ED. Pt w/o anion gap, ketones. VBG was WNL. Low suspicion for DKA/HHS given his work up. Pt with no hx of DM. However, A1C has been increasing over the last year to as high as 6.8 in November 2025. Pt insulin naïve, will plan to initiate modest insulin regimen

Problem List/Past Medical History
Ongoing
- Atrial fibrillation and flutter
- Atrial flutter
- Benign prostatic hyperplasia
- Cardiac defibrillator in situ
- CKD stage 3
- Essential hypertension
- Fall
- G6PD deficiency
- GERD - Gastro-esophageal reflux disease
- Hematoma
- History of pulmonary embolus
- Hyperlipidemia
- Inguinal hernia with incarceration
- Long-term current use of anticoagulant
- Low back pain
- Morbid obesity
- Nonischemic congestive cardiomyopathy
- Open wound of buttock
- Recurrent falls
- Tobacco user
- Warfarin monitoring status
- Weakness

Historical
- No qualifying data

Medications
Inpatient

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0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**				
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Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

Progress Notes

while inpatient. Calculated daily insulin with 0.3 units/kg/day. HbA1c was 11.4% showing uncontrolled diabetes mellitus.
- Start 15 units Glargine nightly, 5 units of Aspart with meals
- Continue bedside glucose monitoring AC/HS
- Cardiac diet

#Acute Kidney Injury
#CKD Stage III
Pt with baseline creatinine 1.6 to 2.1, now at 2.8 on admission. GFR is 23. Last RFP Nov 2025. Pt given 1 L of fluids in the ED.
-Will hold further fluids for now given hx of HFrEF
-Hold home Bumex given new AKI
-Trend renal function daily
-Avoid nephrotoxins
-Renally dose medications

#Urinary Tract Infection
Pt with reported frequency and dysuria. UA was infectious (nitrites, leukocyte esterase, >100 WBC, 4+ bacteria), likely contributing to the pt's increasing weakness. Pt received initial dose of Ceftriaxone in the ED. Will continue abx, f/u cultures.
- Continue IV ceftriaxone, consider transition to oral abx if clinical picture improves
- F/u cultures
- Monitor for clinical response

#Non-Ischemic Cardiomyopathy s/p ICD
#HFrEF
Last Echo July 2024 with EF of 50-55%, with aortic, mitral and tricuspid regurgitation. Pt with very mild LE edema which is now new per the patient. No other signs of volume overload on exam. BNP Mildly elevated. Delta troponins were negative. Will hold further fluids given hx.
- Continue metoprolol succinate 50 mg daily
- Hold bumetanide 2 mg PO daily given AKI
- Strict I/Os
- Consider obtaining Echo Inpatient

#Constipation
Pt with several days hx of constipation, last BM 1-2 days ago. No other GI sx. Mild bilat lower quadrant tenderness on exam. Mild stool burden w/o signs of obstruction on imaging.
- MiraLAX 17 g daily
- CTM

#Hyperkalemia
K 5.4 w/o ECG changes. Likely elevated given hx of CKD and current K supplementation.
- Cardiac monitoring
- Continue Sub Q Insulin
- Hold home potassium chloride 20 mEq daily
- Trend BMP

acetaminophen, 975 mg= 3 tab(s), Oral, TID, PRN
apixaban, 5 mg= 1 tab(s), Oral, BID
cefTRIAXone, 1 g= 50 mL, IV Piggyback (Intravenous), every 24 hr
Crestor, 10 mg= 2 tab(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: for cholesterol ---
Dextrose 50% Sliding Scale, 25-50mL, IV Push (Intravenous), As Directed, PRN , BG less than 54 and unresponsive or NPO: Give 50 mL IV (over 3 min STAT) BG 54-69 and unresponsive or NPO: Give 25 mL IV (over 3 min STAT) Notify provider of hypoglycemia Recheck BG in 15 minutes and provide additional treatment if BG less than 70 Continue to recheck and retreat every 15 minutes until BG greater than 70
finasteride, 5 mg= 1 tab(s), Oral, Daily
folic acid, 1 mg= 1 tab(s), Oral, Daily
glucagon, 1 mg, IntraMuscular, every 15 min, PRN , IF NO IV ACCESS: Glucagon 1mg SubQ STAT for BG of less than 70 mg/dL and unresponsive or NPO. Glucagon may be given IM except for Gvoke (brand) which must be given SQ. Turn patient on left side immediately after medication is administered due to vomiting risks. Notify provider of hypoglycemia and place IV. Recheck BG in 15 minutes. If BG less than 70, may repeat glucagon dosing X 1.
glucose, 15 g= 1 EA, Oral, As Directed, PRN , For BG less than 70 mg/dL and responsive. Notify provider of hypoglycemia. Recheck BG in 15 minutes and provide additional treatment if BG less than 70. Continue to recheck and retreat every 15 minutes until BG greater than 70.
Insulin aspart, 5 unit(s)= 0.05 mL, SubCutaneous, AC (before meals) . - HOLD if meal will be missed (i.e. NPO or refusing to eat) -If pre-meal glucose is 70-110, subtract one unit from this dose

0079A-Mike O'Callaghan Military Medical Center

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Sex/DOB/Age:	Male 1/30/1951 75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365	Veterans ID (ICN):	

Progress Notes

#Atrial Fibrillation / Flutter
Hx of AFIB on Eliquis.
- Continue apixaban 5 mg PO BID
- Continue metoprolol succinate 50 mg PO daily
- Telemetry

#Elevated Troponin
High-sensitivity troponin 32.37 to 32.88. No CP or SOB. Low suspicion for ACS, PE or other acute cardiopulmonary pathology given his work up and exam.
- Continue telemetry

#Open Wound of Buttock
Several small open wounds to buttocks in intergluteal cleft. No purulence, erythema or signs of infection.
- Wound care consult
- Monitor for infection

#Left Inguinal Hernia
No CT evidence of incarceration or obstruction
- Monitor for pain, obstruction symptoms
- Surgical consult if clinical change

#Hypertension
- Continue metoprolol succinate 50 mg PO daily
- Hold bumetanide 2 mg PO daily given AKI
- Monitor BP

#Hyperlipidemia
- Rosuvastatin renally dosed to 10 mg PO daily (home dose 20 mg)
- Pending lipid panel

#Benign Prostatic Hyperplasia
- Continue finasteride 5 mg PO daily

#GERD
- Continue omeprazole 40 mg PO daily

#Chronic Pain
- Continue acetaminophen 1000 mg PO TID PRN pain or fever
- Continue lidocaine 5% patch, 12 hours on / 12 hours off

F: PO
E: Replete PRN
N: Cardiac Diet
GI: Not indicated
DVT: SCD , Home Eliquis
Isolation: none

insulin glargine 100 units/mL subcutaneous solution, 15 unit(s)= 0.15 mL, SubCutaneous, every day at bedtime
Lidoderm 5% topical patch, 1 patch(es), Topical, Daily , Leave on for up to 12 hours within a 24 hour period (12 hours on, 12 hours off)
MiraLax, 17 g, Oral, BID , dissolve in 4-8 ounces of fluid
nicotine 7 mg/24 hr transdermal film, extended release, 1 patch(es), TransDermal, Daily
omeprazole, 40 mg= 2 cap(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: 30 to 60 minutes before a meal ---
ondansetron, 4 mg= 1 tab(s), Oral, every 6 hr, PRN
Toprol-XL, 50 mg= 2 tab(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: for blood pressure —

Home
apixaban 5 mg oral tablet, 5 mg= 1 tab(s), Oral, BID, 3 refills
bumetanide 2 mg oral tablet, 2 mg= 1 tab(s), Oral, Daily
Crestor 20 mg oral tablet, 20 mg= 1 tab(s), Oral, Daily, 3 refills
erythromycin 0.5% eye ointment [3.5g], See Instructions, 3 refills
finasteride 5 mg oral tablet, 5 mg= 1 tab(s), Oral, Daily, 3 refills
Lidoderm 5% topical film, 1 patch(es), Topical, Daily, 3 refills
omeprazole 40 mg oral delayed release capsule, 40 mg= 1 cap(s), Oral, Daily, 3 refills
potassium chloride 20 mEq oral tablet, extended release, 20 mEq= 1 tab(s), Oral, Daily, 3 refills
Toprol-XL 50 mg oral tablet, extended release, 50 mg= 1 tab(s), Oral, Daily, 3 refills
Tylenol Extra Strength 500 mg oral tablet, 1000 mg= 2 tab(s), Oral, TID, PRN, 1 refills

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0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**		
MRN:	3728595000001	Admit:	4/11/2026 21:10 PDT
FIN:	201841085	Discharge:	
Sex/DOB/Age:	Male 1/30/1951 75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365	Veterans ID (ICN):	

Progress Notes

Code: FULL CODE
Dispo: MSU, pending clinical improvement, PT/OT, AKI improvement

Subjective
NAEO. Denies any new or worsening symptoms.

Chief Complaint
HYPERGLYCEMIA

Review of Systems
Constitutional: no fever, no chills
Respiratory: no shortness of breath, no cough
Cardiovascular: no chest pain.
Gastrointestinal: no nausea, no vomiting, mild constipation, no diarrhea, no abdominal pain.
Genitourinary: yes dysuria, yes urinary frequency, nourinary urgency
[1]

Objective

Vitals & Measurements
24 Hour Vitals Range, Ending at 04/13/2026 11:40 (PDT)
·**Temperature Temporal Artery:** 36 - 36.5 Deg C (**min:** 36 Deg C at 19:02, **max:** 36.5 Deg C at 04:34, **latest:** 36.4 Deg C at 08:15)
·**Systolic Blood Pressure:** 122 - 138 mmHg (**min:** 122 mmHg at 04:34, **max:** 138 mmHg at 21:50, **latest:** 127 mmHg at 08:15)
·**Diastolic Blood Pressure:** 69 - 81 mmHg (**min:** 69 mmHg at 04:34, **max:** 81 mmHg at 16:19, **latest:** 77 mmHg at 08:15)
·**Mean Arterial Pressure, Cuff (Calc):** 87 - 99 mmHg (**min:** 87 mmHg at 04:34, **max:** 99 mmHg at 16:19, **latest:** 94 mmHg at 08:15)
·**Peripheral Pulse Rate:** 61 - 65 bpm (**min:** 61 bpm at 21:50, **max:** 65 bpm at 16:19, **latest:** 64 bpm at 08:15)
·**Heart Rate Monitored:** 100 - 100 bpm (**min:** 100 bpm at 04:34, **max:** 100 bpm at 04:34, **latest:** 100 bpm at 04:34)
·**Respiratory Rate:** 16 - 18 br/min (**min:** 16 br/min at 04:34, **max:** 18 br/min at 08:15, **latest:** 18 br/min at 08:15)
·**SpO2:** 96 - 100 % (**min:** 96 % at 08:15, **max:** 100 % at 16:19, **latest:** 96 % at 08:15)

Physical Exam
General: alert, no acute distress. Obese

Respiratory: Lungs CTA , respirations normal work of breathing

Cardiovascular: regular rate and rhythm, murmur absent

Abdomen: abd soft, bowel sounds present , mild RLQ and LLQ tenderness.

Extremities: edema not present, pulses normal Mild edema to bilat LE, left worse than right. Dry skin seen diffusely on pt's LEs and back. Several small open wounds to gluteal cleft, just lateral to anus.

0079A-Mike O'Callaghan Military Medical Center

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Progress Notes

Neurological: oriented x 4 , LOC appropriate for age , moves all extremities, LLE weaker when compared to right (chronic) [2]

[1-2] FMR Admission H&P; JACOBSEN, DYLAN, DO, Family Medicine, Resident 04/12/2026 02:25 PDT

Electronically signed on: 04/13/2026 11:45 PDT

NICHOLAS P SYMONS
MD, Internal Medicine

0079A-Mike O'Callaghan Military Medical Center
4700 North Las Vegas Blvd
Building 1300
Nellis AFB, NV 89191-
(702) 653-3610

Patient Name:	**Downs, Jimmy**	Admit:	4/11/2026 21:10 PDT
MRN:	3728595000001	Discharge:	
FIN:	201841085	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
Sex/DOB/Age:	Male 1/30/1951 75 years	Location:	0079A-INP-MMMSU; 3134 MSU; 1
DOD ID (EDIPI):	1060909365	Veterans ID (ICN):	

Rehabilitation Notes

Document Type:	Physical Therapy Progress Note
Service Date/Time:	4/12/2026 10:00 PDT
Result Status:	Auth (Verified)
Perform Information:	MATHEW, RANI, PT, Physical Therapy (4/12/2026 11:27 PDT)
Sign Information:	MATHEW, RANI, PT, Physical Therapy (4/12/2026 11:27 PDT)

Chief Complaint
HYPERGLYCEMIA

Subjective/History of Present Illness
Subjective:
Chief Complaints: limited mobility
HPI: *75 y/o PMHx of Afib/flutter on apixaban, NICM s/p ICD, CKDIII, HTN, HLD, h/o PE, morbid obesity, recurrent falls, and chronic weakness with baseline limited mobility requiring a walker presents with weakness after a fall from the toilet.*

Consent for Evaluation: nursing and patient agreeable and appropriate for physical therapy evaluation.
Two patient identifiers used for thorough identification: Complete
Patient consents to physical therapy evaluation:yes
Barrier to learning: none

Home environment:
- Patient lives in: one story home
- Steps to Enter: none
- Steps once inside: none
- Lives with: spouse and daughter
- Environment Modification:Walk in shower

DME: front wheeled walker, Four wheeled walker _ _ _ _
Other: shower chair

Prior Level of Function:
- Ambulation: modified independent With front wheeled walker
- ADLs: modified independent

Problem List/Past Medical History
Ongoing
- Atrial fibrillation and flutter
- Atrial flutter
- Benign prostatic hyperplasia
- Cardiac defibrillator in situ
- CKD stage 3
- Essential hypertension
- Fall
- G6PD deficiency
- GERD - Gastro-esophageal reflux disease
- Hematoma
- History of pulmonary embolus
- Hyperlipidemia
- Inguinal hernia with incarceration
- Long-term current use of anticoagulant
- Low back pain
- Morbid obesity
- Nonischemic congestive cardiomyopathy
- Open wound of buttock
- Recurrent falls
- Tobacco user
- Warfarin monitoring status
- Weakness

Historical
- No qualifying data

Procedure/Surgical History
- Anal Fistula Repair

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Rehabilitation Notes

Patient Goals: Get better
Patient comments:
History of falls: yes within six months- the reason for hospitalization. pt denies any other falls.

Pain Scale:
Patient able to quantify pain: yes
Experiencing pain: No. Pt denies any pain at the time of therapy session
Pain tool: Numeric pain rating scale
Location:
Numeric Pain Scale: 0/10
At rest: 0/10 _ During activity: 0/10 _ Post activity:0/10 _
Agg:n/a _ _
Ease:n/a _ _
Pain Descriptors: _
Pain Duration: _
Pain Intervention: _
Patient comments:

Objective:

Pt's response to activity: Patient tolerated functional mobility without any adverse event. Pt denies chest pain or dizziness or lightheadedness or SOB or n/v during the session. RN notified of all. _ _
Observation: Patient is cleared for therapy by RN. Patient found supine in bed with HOB raised and on RA upon arrival. Pt agreeable to therapy session. Pt was able to perform bed mobility, transfer, and gait training without any adverse event. Pt was not in acute distress. Pt denies chest pain or dizziness or lightheadedness or SOB or n/v during the session.

Oriented to: A & O x 4 (person, place, time, circumstance)
Communication: Fluent
Skin:Warm and dry, intact on visible area

ROM:
UE Gross AROM/PROM: within functional limits
LE Gross AROM/PROM: within functional limits

Strength:
Upper Extremity:within functional limits
Lower Extremity: RLE grossly 4/5; L LE grossly 3-/5

Therapeutic Exercises performed: NA

Neurological status: Sensation: light touch sensation intact

Functional Assessment:

- Bilateral Bunionectomy
- Hernia Repair
- Left Breast Lumpectomy

Medications

acetaminophen, 975 mg= 3 tab(s), Oral, TID, PRN
apixaban, 5 mg= 1 tab(s), Oral, BID
apixaban 5 mg oral tablet, 5 mg= 1 tab(s), Oral, BID, 3 refills
bumetanide 2 mg oral tablet, 2 mg= 1 tab(s), Oral, Daily
cefTRIAXone, 1 g= 50 mL, IV Piggyback (Intravenous), every 24 hr
Crestor, 10 mg= 2 tab(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: for cholesterol ---
Crestor 20 mg oral tablet, 20 mg= 1 tab(s), Oral, Daily, 3 refills
Dextrose 50% Sliding Scale, 25-50mL, IV Push (Intravenous), As Directed, PRN , BG less than 54 and unresponsive or NPO: Give 50 mL IV (over 3 min STAT) BG 54-69 and unresponsive or NPO: Give 25 mL IV (over 3 min STAT) Notify provider of hypoglycemia Recheck BG in 15 minutes and provide additional treatment if BG less than 70 Continue to recheck and retreat every 15 minutes until BG greater than 70
erythromycin 0.5% eye ointment [3.5g], See Instructions, 3 refills
finasteride, 5 mg= 1 tab(s), Oral, Daily
finasteride 5 mg oral tablet, 5 mg= 1 tab(s), Oral, Daily, 3 refills
folic acid, 1 mg= 1 tab(s), Oral, Daily
glucagon, 1 mg, IntraMuscular, every 15 min, PRN , IF NO IV ACCESS: Glucagon 1mg SubQ STAT for BG of less than 70 mg/dL and unresponsive or NPO. Glucagon may be given IM except for Gvoke (brand) which must be given SQ. Turn patient on left side immediately after medication is administered due to vomiting risks. Notify provider of hypoglycemia and place IV. Recheck BG in 15 minutes. If BG less than 70, may repeat glucagon dosing X 1.

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Rehabilitation Notes

Bed Mobility
Rolling:Modified independent
Scooting:Modified independent
Supine to sit:supervision
Sit to supine: supervision

Transfer:
Sit to stand:Contact Guard/Min assist with FWW from standard height bed- 3 rep noted difficulty performing rockign back and forth three times.
Stand to sit:Contact Guard with FWW - 3 rep
Bed to chair:Minimal Assist x 1 with use of arm rest
Chair to bed:Minimal Assist x 1 with use of arm rest
Balance:
Sitting: Static: Great
Dynamic: Great
Standing: Static: Good
Dynamic: Fair-
Kansas University Balance Scale:
Standing: ~2+/5

Gait Assessment:
Deviations: left knee buckle noted- a couple of time
Assistive Device:front wheeled walker
Distance: ~100ft
Level of Assistance: Contact Guard/min assist for safety
Stairs: n/a

Outcome measures: n/a

Patient positioning post physical therapy session: supine in bed with head of bed raised, Purewick intact, on RA, nursing in the room, call light within the reach, RN notified of all.

Assessment:

PT diagnosis: decreased mobility, gait abnormalities, difficulty with transfers, decreased endurance
Clinical Assessment: Patient presents with decreased out of bed mobility, LE weakness, decreased endurance, decreased functional strength, difficulty with bed mobility, transfer, and walking secondary to deconditioning from medical conditions. Patient would benefit from skilled PT to increase out of bed mobility and to increase overall strength during the hospital stay. Recommend home with family assist and home health PT for home transitioning and to improve functional independents.
Rehab Potential:Good

Plan:
Frequency: 5-7x/week

glucose, 15 g= 1 EA, Oral, As Directed, PRN , For BG less than 70 mg/dL and responsive. Notify provider of hypoglycemia. Recheck BG in 15 minutes and provide additional treatment if BG less than 70. Continue to recheck and retreat every 15 minutes until BG greater than 70.
insulin aspart, 5 unit(s)= 0.05 mL, SubCutaneous, AC (before meals) , -HOLD if meal will be missed (i.e. NPO or refusing to eat) -If pre-meal glucose is 70-110, subtract one unit from this dose
insulin glargine 100 units/mL subcutaneous solution, 15 unit(s)= 0.15 mL, SubCutaneous, every day at bedtime
Lidoderm 5% topical film, 1 patch(es), Topical, Daily, 3 refills
Lidoderm 5% topical patch, 1 patch(es), Topical, Daily , Leave on for up to 12 hours within a 24 hour period (12 hours on, 12 hours off)
MiraLax, 17 g, Oral, BID , dissolve in 4-8 ounces of fluid
nicotine 7 mg/24 hr transdermal film, extended release, 1 patch(es), TransDermal, Daily
omeprazole, 40 mg= 2 cap(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: 30 to 60 minutes before a meal ---
omeprazole 40 mg oral delayed release capsule, 40 mg= 1 cap(s), Oral, Daily, 3 refills
ondansetron, 4 mg= 1 tab(s), Oral, every 6 hr, PRN
potassium chloride 20 mEq oral tablet, extended release, 20 mEq= 1 tab(s), Oral, Daily, 3 refills
Toprol-XL, 50 mg= 2 tab(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: for blood pressure ---
Toprol-XL 50 mg oral tablet, extended release, 50 mg= 1 tab(s), Oral, Daily, 3 refills
Tylenol Extra Strength 500 mg oral tablet, 1000 mg= 2 tab(s), Oral, TID, PRN, 1 refills

Allergies
aspirin (Unknown)

Social History
Alcohol
Frequency: Daily. Average drinks per episode in last year: 2., 06/27/2024

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0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**				
MRN:	3728595000001			Admit:	4/11/2026 21:10 PDT
FIN:	201841085			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

Rehabilitation Notes

Precautions: high fall risk- left knee buckle
Special instructions: Nursing staff to ambulate with pt. 1-2x daily as tolerated with front wheeled walker and with assist. Please follow behind with w/c for safety

Treatment activities: Bed Mobility / Transfer Training (97530); Therapeutic Exercises (97110); Gait / Stair Training (97116); Neuromuscular Reeducation / Balance (97112); Patient / Family Education (98960)

Discharge goals:
1. Patient will be able to ambulate ~100 feet x 2 with Stand-by assist with front wheeled walker assist, no loss of balance, and no verbal cues to maintain safety within 3-4 days.
2. Patient will be able to perform all functional transfers with Stand-by assist with front wheeled walker assist and no verbal cues to maintain safety within 3-4 days.
3. Patient will be able to perform bed mobility at Modified Independent assist and no verbal cues within 3-4 days

Recommended DME: raised toilet seat
Discharge recommendation: home with family assist and home health physical therapy upon discharge.
Other/staff communication: RN
Patient or family teaching:POC, discharge recommendations, safety awareness, fall risk precautions, gait training, transfer, bed mobility, energy conservation, issued a disposable safety belt.
Evaluation/Treatment:Physical Therapy evaluation: Low complexity 97161

//signed
Mathew, Rani, PT
Staff Physical Therapist
99th Medical Group, OMRS
Mike O'Callaghan Military Medical Center
Nellis AFB, NV

Objective/Physical Exam

Vitals & Measurements
HR: 63(Peripheral) **RR:** 18 **BP:** 141/78 **SpO2:** 98%

Current (Last 12 months) Use:. Liquor Type:. Daily Frequency:. 4-5 drinks/day Average drinks per episode in last year:., 07/13/2022
Beer, Liquor, Daily, 12/01/2020

Employment/School
Status: Retired., 07/09/2025

Tobacco
Frequent/Daily exposure to secondhand smoke in indoor/confined spaces Yes. Cigarette use: Yes-current everyday cigarette user. Average PACKS per day: (10 cigarettes = 0.5 packs) 0.5. Other Tobacco use: Never-other tobacco user (not cigarettes)., 06/28/2025
Frequent/Daily exposure to secondhand smoke in indoor/confined spaces No. Cigarette use: Never-cigarette user. Other Tobacco use: Never-other tobacco user (not cigarettes)., 06/04/2025
Exposure to Secondhand Smoke: No. Yes-current everyday cigarette user Cigarette use:. 0.5 Average PACKS per day: (10 cigarettes = 0.5 packs). Never-other tobacco user (not cigarettes) Other Tobacco use:., 07/13/2022
Yes-current everyday cigarette user Cigarette use:. 0.25 Average PACKS per day: (10 cigarettes = 0.5 packs). Never-other tobacco user (not cigarettes) Other Tobacco use:., 06/29/2022
Yes-current everyday cigarette user Cigarette use:. 0.25 Average PACKS per day: (10 cigarettes = 0.5 packs). Never-other tobacco user (not cigarettes) Other Tobacco use:., 04/01/2022
Smoking tobacco use: 10 or more ciga(1/2 pack or more)/day in last 30 days. Type: Cigarettes., 06/04/2021

Family History
No family history documented

Electronically signed on: 04/12/2026 11:27 PDT

RANI MATHEW
PT, Physical Therapy

Unless you are the subject of the record, protections under 42 CFR Part 2 and/or 38 U.S.C. 7332 may apply.

0079A-Mike O'Callaghan Military Medical Center
4700 North Las Vegas Blvd
Building 1300
Nellis AFB, NV 89191-
(702) 653-3610

Patient Name:	**Downs, Jimmy**	Admit:	4/11/2026 21:10 PDT
MRN:	3728595000001	Discharge:	
FIN:	201841085	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
Sex/DOB/Age:	Male 1/30/1951 75 years	Location:	0079A-INP-MMMSU; 3134 MSU; 1
DOD ID (EDIPI):	1060909365	Veterans ID (ICN):	

History and Physical Reports

Document Type:	Hosp Admit H+P Note
Service Date/Time:	4/12/2026 02:25 PDT
Result Status:	Modified
Perform Information:	JACOBSEN, DYLAN, DO, Family Medicine, Resident (4/12/2026 02:26 PDT)
Sign Information:	ROLDANPROVIDEL, CHRISTINA R, MD, Family Medicine, Resident (4/12/2026 06:17 PDT); JACOBSEN, DYLAN, DO, Family Medicine, Resident (4/12/2026 05:37 PDT); JACOBSEN, DYLAN, DO, Family Medicine, Resident (4/12/2026 02:26 PDT)

CHIEF COMPLAINT
HYPERGLYCEMIA

HISTORY OF PRESENT ILLNESS
75 y/o PMHx of Afib/flutter on apixaban, NICM s/p ICD, CKDIII, HTN, HLD, h/o PE, morbid obesity, recurrent falls, and chronic weakness with baseline limited mobility requiring a walker presents with weakness after a fall from the toilet. Earlier today he was on the toilet for about 4 hours due to constipation, last BM 1-2 days prior. During episode of prolonged sitting on the toilet, his legs became weak and he fell off the toilet and was unable to get up. Pt states that he and his wife then called an ambulance and was transported to the hospital when his sugars were found to be in the 500s. . He denied LOC, CP, SOB, Fevers. Pt says that he just generally feels tired, states not any worse than his normal baseline weakness. He also reports dysuria and urinary frequency over the last several days. Pt denies hx of DM but says that he used to take medication for blood sugar a long time ago and does not remember the medication name. Pt also complains of some rectal wounds which he says his wife helps him maintain. Pt states that he lives at home with his wife and daughter. Pt reports 0.5 PPD smoking hx since the age of 9, drinks 1-2 beers daily.

ED Course:
- Vitals: within normal limits

- Labs:
Labs (Last four charted values)
WBC 8.68 (APR 11)
Hgb 13.8 (APR 11)

Radiology Results

CT Brain/Head w/o Contrast

04/11/26 20:54:03
IMPRESSION:
1. Probable small vessel ischemic changes seen in the superior ventricular and basal ganglia regions bilaterally not significantly changed from the comparison study.
2. No evidence of intracranial hemorrhage or mass.
3. Cerebral atrophy unremarkable for the patient's age.
4. Probable mild chronic ethmoid, sphenoid, and maxillary sinusitis.
5. Mild soft tissue prominence in the frontal regions. No underlying fractures are evident.

Electronically Signed by : Dr. WILLIAM BURGE
Approving Radiologist: WILLIAM BURGE NPI: 1669503090

This information has been disclosed to you from federal health records in accordance with an approved written request. Unless you are the subject of the record, federal laws may prohibit you from making a further disclosure of this record for a purpose other than why the information was provided to you. If this record contains substance abuse information, it is protected by 42 CFR Part 2 and/or 38 U.S.C. 7332 and the redisclosure restrictions of those federal laws on individuals who are not the subject of the record apply.

0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**		
MRN:	3728595000001	Admit:	4/11/2026 21:10 PDT
FIN:	201841085	Discharge:	
Sex/DOB/Age:	Male 1/30/1951 75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365	Veterans ID (ICN):	

History and Physical Reports

Hct 43.6 (APR 11)
Plt 236 (APR 11)
Na L **135** (APR 11)
K H **5.4** (APR 11)
CO2 22.3 (APR 11)
Cl 104 (APR 11)
Cr H **2.79** (APR 11)
BUN H **24** (APR 11)
Glucose C **556** (APR 11)- > 342
Mg 2.3 (APR 11)
Ca 9.1 (APR 11)
PT 11.5 (APR 11)
INR H **1.11** (APR 11)
BNP: 245

Delta trop negative.
TSH: 1.238
Ethanol <3.
Urine with bacteria , WBCs, nitrites. LE.

- Rads:

CT Brain/Head w/o Contrast

04/11/26 20:54:03
IMPRESSION:
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2. No evidence of intracranial hemorrhage or mass.
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5. Mild soft tissue prominence in the frontal regions. No underlying fractures are evident.

Electronically Signed by : Dr. WILLIAM BURGE
Approving Radiologist: WILLIAM BURGE NPI: 1669503090

CODE 1000 CODE 1000 NO ALERT REQUIRED
IMAGE COUNT: 1128
Report is based on imaging and clinical information provided by the facility at time of interpretation. Prior imaging and reports may not be available.

This report was dictated using Powerscribe 360 and may contain transcription errors despite proofreading. Please interpret statements in context and contact the radiologist if there are any questions.

ATTENTION: PATIENTS - If you have questions or concerns about the results in this report, please contact your ordering provider or primary care team.

CODE 1000 CODE 1000 NO ALERT REQUIRED
IMAGE COUNT: 1128
Report is based on imaging and clinical information provided by the facility at time of interpretation. Prior imaging and reports may not be available.

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ATTENTION: PATIENTS - If you have questions or concerns about the results in this report, please contact your ordering provider or primary care team.

Signed By: BURGE, WILLIAM

****

CT Spine Cervical w/o Contrast

04/11/26 20:54:05
IMPRESSION:

No evidence of acute fracture or significant subluxation cervical spine, with findings as above.

Electronically Signed by : Dr. Ernest Kinchen, MD
Approving Radiologist: Ernest Kinchen NPI: 1265415764

CODE 2 CODE 2 MINOR ABNORMALITY
IMAGE COUNT: 1003
Report is based on imaging and clinical information provided by the facility at time of interpretation. Prior imaging and reports may not be available.

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0079A-Mike O'Callaghan Military Medical Center

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FIN:	201841085			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

Signed By: BURGE, WILLIAM

****

CT Spine Cervical w/o Contrast

04/11/26 20:54:05
IMPRESSION:

No evidence of acute fracture or significant subluxation cervical spine, with findings as above.

Electronically Signed by : Dr. Ernest Kinchen, MD
Approving Radiologist: Ernest Kinchen NPI: 1265415764

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ATTENTION: PATIENTS - If you have questions or concerns about the results in this report, please contact your ordering provider or primary care team.

Signed By: KINCHEN III, ERNEST W

****

CT Chest/Abdomen/Pelvis w/ Contrast

04/11/26 21:02:06
IMPRESSION: No evidence of acute fracture or dislocation.
2. Mild COPD/edematous changes. Atelectasis and/or fibrosis. No evidence of pneumothorax, pneumomediastinum, pulmonary parenchymal contusion or pleural effusion.
3. Large left inguinal hernia containing portions of sigmoid colon again demonstrated without secondary bowel obstruction or wall thickening of the contained loops. Mild to moderate scattered feces colon. No evidence of bowel obstruction. Visualized appendix within normal limits.
4. No significant, acute abnormalities otherwise, with findings as above. Follow up as clinically indicated.

Electronically Signed by : Dr. Ernest Kinchen, MD
Approving Radiologist: Ernest Kinchen NPI: 1265415764

CODE 2 CODE 2 MINOR ABNORMALITY
IMAGE COUNT: 739

ATTENTION: PATIENTS - If you have questions or concerns about the results in this report, please contact your ordering provider or primary care team.

Signed By: KINCHEN III, ERNEST W

****

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04/11/26 21:02:06
IMPRESSION: No evidence of acute fracture or dislocation.
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Electronically Signed by : Dr. Ernest Kinchen, MD
Approving Radiologist: Ernest Kinchen NPI: 1265415764

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Report is based on imaging and clinical information provided by the facility at time of interpretation. Prior imaging and reports may not be available.

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ATTENTION: PATIENTS - If you have questions or concerns about the results in this

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DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

Report is based on imaging and clinical information provided by the facility at time of interpretation. Prior imaging and reports may not be available.

This report was dictated using Powerscribe 360 and may contain transcription errors despite proofreading. Please interpret statements in context and contact the radiologist if there are any questions.

ATTENTION: PATIENTS - If you have questions or concerns about the results in this report, please contact your ordering provider or primary care team.

Signed By: KINCHEN III, ERNEST W

- Treatments: Tylenol, IV fluids, insulin

Allergies:
- aspirin

Meds:
Medication List
Active Medications
Ordered
acetaminophen: 1,000 mg, Oral, TID, PRN: pain, mild (DVPRS, 1-4).
apixaban: 5 mg, Oral, BID.
dextrose 50% in water: 25-50mL, IV Push (Intravenous), As Directed, PRN: hypoglycemia (see comment).
finasteride: 5 mg, Oral, Daily.
glucagon: 1 mg, IntraMuscular, every 15 min, PRN: other (see comment).
glucose: 15 g, Oral, As Directed, PRN: hypoglycemia (see comment).
insulin aspart: 5 unit(s), SubCutaneous, AC (before meals).
insulin glargine: 15 unit(s) = 0.15 mL, SubCutaneous, every day at bedtime.
lidocaine topical: 1 patch(es), Topical, Daily.
metoprolol: 50 mg, Oral, Daily.
omeprazole: 40 mg, Oral, Daily.
rosuvastatin: 20 mg, Oral, Daily.
Prescribed
acetaminophen: 2 tab(s), Oral, TID, PRN: pain or fever, 200 tab(s), 1 Refill(s).
apixaban: 1 tab(s), Oral, BID, 180 tab(s), 3 Refill(s).
apixaban: 1 tab(s), Oral, BID, 180 EA, 3 Refill(s).
erythromycin ophthalmic: See Instructions, 3.5 g, 3 Refill(s).
finasteride: 1 tab(s), Oral, Daily, 90 tab(s), 3 Refill(s).
finasteride: 1 tab(s), Oral, Daily, 90 EA, 3 Refill(s).
lidocaine topical: 1 patch(es), Topical, Daily, Leave on for up to 12 hours within a 24 hour period (12 hours on, 12 hours off), 30 patch(es), 3 Refill(s).
metoprolol: 1 tab(s), Oral, Daily, for blood pressure, 90 tab(s), 3 Refill(s).
metoprolol succinate: 1 tab(s), Oral, Daily, for blood pressure., 90

report, please contact your ordering provider or primary care team.

Signed By: KINCHEN III, ERNEST W

PROBLEM LIST/PAST MEDICAL HISTORY
Ongoing
Atrial fibrillation and flutter
Atrial flutter
Benign prostatic hyperplasia
Cardiac defibrillator in situ
CKD stage 3
Essential hypertension
Fall
G6PD deficiency
GERD - Gastro-esophageal reflux disease
Hematoma
History of pulmonary embolus
Hyperlipidemia
Inguinal hernia with incarceration
Long-term current use of anticoagulant
Low back pain
Morbid obesity
Nonischemic congestive cardiomyopathy
Open wound of buttock
Recurrent falls
Tobacco user
Warfarin monitoring status
Weakness
Historical
No qualifying data

PROCEDURE/SURGICAL HISTORY
- Anal Fistula Repair
- Bilateral Bunionectomy
- Hernia Repair
- Left Breast Lumpectomy

MEDICATIONS
Inpatient
acetaminophen, 975 mg= 3 tab(s), Oral, TID, PRN
apixaban, 5 mg= 1 tab(s), Oral, BID
Crestor, 10 mg= 2 tab(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: for cholesterol ---
Dextrose 50% Sliding Scale, 25-50mL, IV Push (Intravenous), As Directed, PRN ,

0079A-Mike O'Callaghan Military Medical Center

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History and Physical Reports

EA, 3 Refill(s).
omeprazole: 1 cap(s), Oral, Daily, 30 to 60 minutes before a meal, 90 cap(s), 3 Refill(s).
omeprazole: 1 cap(s), Oral, 30 to 60 minutes before a meal, 90 EA, 3 Refill(s).
potassium chloride: 1 tab(s), Oral, Daily, 90 tab(s), 3 Refill(s).
potassium chloride: 1 tab(s), Oral, Daily, 90 EA, 3 Refill(s).
rosuvastatin: 1 tab(s), Oral, Daily, for cholesterol, 90 tab(s), 3 Refill(s).
rosuvastatin: 1 tab(s), Oral, Daily, for cholesterol, 90 EA, 3 Refill(s).

Documented
bumetanide: 1 tab(s), Oral, Daily, 30 tab(s), 0 Refill(s).

Medications Inactivated in the Last 72 Hours
acetaminophen: 1,000 mg = 100 mL, 400 mL/hr, IV Piggyback (Intravenous), Once.
cefTRIAXone: 1 g = 50 mL, 100 mL/hr, IV Piggyback (Intravenous), Once.
enoxaparin: 40 mg, SubCutaneous, Daily.
insulin regular: 10 unit(s) = 0.1 mL, IV Push (Intravenous), Once.
Iohexol: 100 mL, IV Push (Intravenous), Once.
lactated ringers injection: 1,000 mL, 2000 mL/hr, IV Bolus (Intravenous), Once.

PMHx: _
Active Problems (22)
Atrial fibrillation and flutter
Atrial flutter
Benign prostatic hyperplasia
Cardiac defibrillator in situ
CKD stage 3
Essential hypertension
Fall
G6PD deficiency
GERD - Gastro-esophageal reflux disease
Hematoma
History of pulmonary embolus
Hyperlipidemia
Inguinal hernia with incarceration
Long-term current use of anticoagulant
Low back pain
Morbid obesity
Nonischemic congestive cardiomyopathy
Open wound of buttock
Recurrent falls
Tobacco user
Warfarin monitoring status
Weakness

PSHx: _

BG less than 54 and unresponsive or NPO: Give 50 mL IV (over 3 min STAT) BG 54-69 and unresponsive or NPO: Give 25 mL IV (over 3 min STAT) Notify provider of hypoglycemia Recheck BG in 15 minutes and provide additional treatment if BG less than 70 Continue to recheck and retreat every 15 minutes until BG greater than 70
diazePAM, 5 mg= 1 tab(s), Oral, every 1 hr, PRN , For CIWA-Ar score greater than/equal to 10 May give additional 5mg dose if needed after 10 min
diazePAM, 10 mg= 2 tab(s), Oral, every 1 hr, PRN , For CIWA-Ar score greater than/equal to 10 if initial 5mg dose is ineffective.
finasteride, 5 mg= 1 tab(s), Oral, Daily
folic acid, 1 mg= 1 tab(s), Oral, Daily
glucagon, 1 mg, IntraMuscular, every 15 min, PRN , IF NO IV ACCESS: Glucagon 1mg SubQ STAT for BG of less than 70 mg/dL and unresponsive or NPO, Glucagon may be given IM except for Gvoke (brand) which must be given SQ. Turn patient on left side immediately after medication is administered due to vomiting risks. Notify provider of hypoglycemia and place IV. Recheck BG in 15 minutes. If BG less than 70, may repeat glucagon dosing X 1.
glucose, 15 g= 1 EA, Oral, As Directed, PRN , For BG less than 70 mg/dL and responsive. Notify provider of hypoglycemia. Recheck BG in 15 minutes and provide additional treatment if BG less than 70. Continue to recheck and retreat every 15 minutes until BG greater than 70.
insulin aspart, 5 unit(s)= 0.05 mL, SubCutaneous, AC (before meals) , - HOLD if meal will be missed (i.e. NPO or refusing to eat) -If pre-meal glucose is 70-110, subtract one unit from this dose
insulin glargine 100 units/mL subcutaneous solution, 15 unit(s)= 0.15 mL, SubCutaneous, every day at bedtime
Lidoderm 5% topical patch, 1 patch(es), Topical, Daily , Leave on for up to 12

0079A-Mike O'Callaghan Military Medical Center

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History and Physical Reports

Social Hx
- EtOH use: Daily
- Tobacco use: None
- Recreational Drugs: None

Family Contact: Ana Downs, wife, 702-587-3876

Code Status: Full Code

REVIEW OF SYSTEMS
Constitutional: no fever, no chills
Respiratory: no shortness of breath, no cough
Cardiovascular: no chest pain.
Gastrointestinal: no nausea, no vomiting, mild constipation, no diarrhea, no abdominal pain.
Genitourinary: yes dysuria, yes urinary frequency, nourinary urgency

PHYSICAL EXAM

Vitals & Measurements
24 Hour Vitals Range, Ending at 04/12/2026 02:25 (PDT)
·**Temperature Temporal Artery:** 36.2 - 36.8 Deg C (**min:** 36.2 Deg C at 01:12, **max:** 36.8 Deg C at 21:21, **latest:** 36.2 Deg C at 01:12)
·**Systolic Blood Pressure:** 122 - 148 mmHg (**min:** 122 mmHg at 21:21, **max:** 148 mmHg at 22:00, **latest:** 138 mmHg at 01:12)
·**Diastolic Blood Pressure:** 71 - 90 mmHg (**min:** 71 mmHg at 21:21, **max:** 90 mmHg at 23:08, **latest:** 75 mmHg at 01:12)
·**Mean Arterial Pressure, Cuff (Calc):** 96 - 99 mmHg (**min:** 96 mmHg at 01:12, **max:** 99 mmHg at 22:00, **latest:** 96 mmHg at 01:12)
·**Peripheral Pulse Rate:** 55 - 77 bpm (**min:** 55 bpm at 21:21, **max:** 77 bpm at 23:35, **latest:** 68 bpm at 01:12)
·**Heart Rate Monitored:** 78 - 78 bpm (**min:** 78 bpm at 22:00, **max:** 78 bpm at 22:00, **latest:** 78 bpm at 22:00)
·**Respiratory Rate:** 14 - 18 br/min (**min:** 14 br/min at 21:21, **max:** 18 br/min at 01:12, **latest:** 18 br/min at 01:12)
·**SpO2:** 98 - 100 % (**min:** 98 % at 23:35, **max:** 100 % at 23:08, **latest:** 99 % at 01:12)

General: alert, no acute distress. Obese

Respiratory: Lungs CTA , respirations normal work of breathing

Cardiovascular: regular rate and rhythm, murmur absent

Abdomen: abd soft, bowel sounds present , mild RLQ and LLQ tenderness.

Extremities: edema not present, pulses normal Mild edema to bilat LE, left worse than right. Dry skin seen diffusely on pt's LEs and back. Several small open wounds to gluteal cleft, just lateral to anus.

hours within a 24 hour period (12 hours on, 12 hours off)
MiraLax, 17 g, Oral, BID , dissolve in 4-8 ounces of fluid
nicotine 7 mg/24 hr transdermal film, extended release, 1 patch(es), TransDermal, Daily
omeprazole, 40 mg= 2 cap(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: 30 to 60 minutes before a meal ---
ondansetron, 4 mg= 1 tab(s), Oral, every 6 hr, PRN
thiamine, 100 mg= 1 tab(s), Oral, Daily
Toprol-XL, 50 mg= 2 tab(s), Oral, Daily , --- At home, patient was taking medication with the following details: Special Instructions: for blood pressure ---

Home
apixaban 5 mg oral tablet, 5 mg= 1 tab(s), Oral, BID, 3 refills
bumetanide 2 mg oral tablet, 2 mg= 1 tab(s), Oral, Daily
Crestor 20 mg oral tablet, 20 mg= 1 tab(s), Oral, Daily, 3 refills
erythromycin 0.5% eye ointment [3.5g], See Instructions, 3 refills
finasteride 5 mg oral tablet, 5 mg= 1 tab(s), Oral, Daily, 3 refills
Lidoderm 5% topical film, 1 patch(es), Topical, Daily, 3 refills
omeprazole 40 mg oral delayed release capsule, 40 mg= 1 cap(s), Oral, Daily, 3 refills
potassium chloride 20 mEq oral tablet, extended release, 20 mEq= 1 tab(s), Oral, Daily, 3 refills
Toprol-XL 50 mg oral tablet, extended release, 50 mg= 1 tab(s), Oral, Daily, 3 refills
Tylenol Extra Strength 500 mg oral tablet, 1000 mg= 2 tab(s), Oral, TID, PRN, 1 refills

QUALITY MEASURES
VTE Form Incomplete, or not indicated. See medications.

0079A-Mike O'Callaghan Military Medical Center

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DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

Neurological: oriented x 4 , LOC appropriate for age , moves all extremities. LLE weaker when compared to right (chronic)

ASSESSMENT/PLAN

Atrial fibrillation and flutter
Ordered:
rosuvastatin (Crestor), 10 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT

Atrial flutter
Ordered:
rosuvastatin (Crestor), 10 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT

Benign prostatic hyperplasia

Cardiac defibrillator in situ
Ordered:
rosuvastatin (Crestor), 10 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT

CKD stage 3

Essential hypertension
Ordered:
rosuvastatin (Crestor), 10 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT

Fall

GERD - Gastro-esophageal reflux disease

History of pulmonary embolus
Ordered:
rosuvastatin (Crestor), 10 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT

Hyperglycemia, unspecified

Hyperkalemia

Hyperlipidemia

Inguinal hernia

Left inguinal hernia

Medication refill
Ordered:
omeprazole (omeprazole), 40 mg, Oral, Capsule-Delayed Release, Daily, First Dose: 04/12/2026 07:30:00 PDT

Nonischemic congestive cardiomyopathy

ALLERGIES
aspirin (Unknown)

SOCIAL HISTORY

Alcohol
Frequency: Daily. Average drinks per episode in last year: 2., 06/27/2024
Current (Last 12 months) Use:. Liquor Type:. Daily Frequency:. 4-5 drinks/day Average drinks per episode in last year:., 07/13/2022.
Beer, Liquor, Daily, 12/01/2020

Employment/School
Status: Retired., 07/09/2025

Tobacco
Frequent/Daily exposure to secondhand smoke in indoor/confined spaces Yes. Cigarette use: Yes-current everyday cigarette user. Average PACKS per day: (10 cigarettes = 0.5 packs) 0.5. Other Tobacco use: Never-other tobacco user (not cigarettes)., 06/28/2025
Frequent/Daily exposure to secondhand smoke in indoor/confined spaces No. Cigarette use: Never-cigarette user. Other Tobacco use: Never-other tobacco user (not cigarettes)., 05/04/2025
Exposure to Secondhand Smoke: No. Yes-current everyday cigarette user Cigarette use:. 0.5 Average PACKS per day: (10 cigarettes = 0.5 packs). Never-other tobacco user (not cigarettes) Other Tobacco use:., 07/13/2022
Yes-current everyday cigarette user Cigarette use:. 0.25 Average PACKS per day: (10 cigarettes = 0.5 packs). Never-other tobacco user (not cigarettes) Other Tobacco use:., 06/29/2022
Yes-current everyday cigarette user Cigarette use:. 0.25 Average PACKS per day: (10 cigarettes = 0.5 packs). Never-other tobacco user (not cigarettes) Other Tobacco use:., 04/01/2022
Smoking tobacco use: 10 or more cigs(1/2 pack or more)/day in last 30 days. Type: Cigarettes., 06/04/2021

FAMILY HISTORY
No family history documented

0079A-Mike O'Callaghan Military Medical Center

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FIN:	201841085			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

Open wound of buttock

Serum creatinine raised

Urinary tract infection, site not specified

Weakness
Ordered:
Occupational Therapy (OT) Inpatient Evaluation and Treatment
Physical Therapy (PT) Inpatient Evaluation and Treatment

Weakness

Well adult exam
Ordered:
apixaban (apixaban), 5 mg, Oral, Tablet, BID, First Dose: 04/12/2026 10:00:00 PDT, 04/12/2026 00:16:00 PDT
finasteride (finasteride), 5 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT, Hazardous, 04/12/2026 00:16:00 PDT
lidocaine topical (Lidoderm 5% topical patch), 1 patch(es), Topical, Patch, Daily, First Dose: 04/12/2026 09:00:00 PDT

Orders:
acetaminophen (acetaminophen), 975 mg, Oral, Tablet, TID, PRN pain, mild (DVPRS, 1-4), First Dose: 04/12/2026 00:16:00 PDT
dextrose 50% in water (Dextrose 50% Sliding Scale), 25-50mL, IV Push (intravenous), Form: Injection, As Directed PRN hypoglycemia (see comment), First Dose: 04/12/2026 00:09:00 PDT
diazePAM (diazePAM), 10 mg, Oral, Tablet, every 1 hr, PRN other (see comment), Order Duration: 7 days, First Dose: 04/12/2026 00:26:00 PDT, Stop Date: 04/19/2026 00:25:00 PDT
diazePAM (diazePAM), 5 mg, Oral, Tablet, every 1 hr, PRN other (see comment), Order Duration: 7 days, First Dose: 04/12/2026 00:26:00 PDT, Stop Date: 04/19/2026 00:25:00 PDT
folic acid (folic acid), 1 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT, 04/12/2026 00:26:00 PDT
glucagon (glucagon), 1 mg, IntraMuscular, Injection, every 15 min, PRN other (see comment), Order Duration: 2 dose(s), First Dose: 04/12/2026 00:09:00 PDT, Stop Date: Limited # of times, 04/12/2026 00:09:00 PDT
glucose (glucose), 15 g, Oral, Gel-Oral, As Directed, PRN hypoglycemia (see comment), First Dose: 04/12/2026 00:09:00 PDT, 04/12/2026 00:09:00 PDT
insulin aspart (insulin aspart), 5 unit(s), SubCutaneous, Injection-Insulin, AC (before meals), First Dose: 04/12/2026 00:09:00 PDT, 04/12/2026 00:09:00 PDT
insulin glargine (insulin glargine 100 units/mL subcutaneous solution), 15 unit(s), SubCutaneous, Injection-Insulin, every day at bedtime, First Dose: 04/12/2026 00:09:00 PDT, 04/12/2026 00:09:00 PDT
polyethylene glycol 3350 (MiraLax), 17 g, Oral, Powder-Oral-Gram, BID, First Dose: 04/12/2026 10:00:00 PDT, 04/12/2026 01:09:00 PDT

0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**				
MRN:	3728595000001			Admit:	4/11/2026 21:10 PDT
FIN:	201841085			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

nicotine (nicotine 7 mg/24 hr transdermal film, extended release), 1 patch(es), TransDermal, Patch, Daily, First Dose: 04/12/2026 09:00:00 PDT, 04/12/2026 00:27:00 PDT
ondansetron (ondansetron), 4 mg, Oral, Tablet-Disintegrating, every 6 hr, PRN nausea, First Dose: 04/12/2026 00:26:00 PDT, 04/12/2026 00:26:00 PDT
thiamine (thiamine), 100 mg, Oral, Tablet, Daily, First Dose: 04/12/2026 09:00:00 PDT, 04/12/2026 00:26:00 PDT
metoprolol (Toprol-XL), 50 mg, Oral, Tablet-Extended Release, Daily, First Dose: 04/12/2026 09:00:00 PDT, 04/12/2026 00:16:00 PDT
Regular Diet
Albumin/Creatinine Ratio, Urine
Basic Metabolic Panel
CBC w/ Diff
Hemoglobin A1c
Lipid Panel
Magnesium Level
Phosphorus Level
*Resuscitation Status
Ambulate
Cardiac Monitoring
CIWA-Ar Scoring
Communication Order
Fall Risk Precautions
Neurological Checks
Notify Treating Provider
Notify Treating Provider Vital Signs
Peripheral IV Insertion
POC Glucose Level
Smoking Cessation Instruction
Up with Assistance
Vital Signs
Weight
Oxygen Therapy

75 y/o PMHx of Afib/flutter on apixaban, NICM s/p ICD, HFrEF, CKDIII, HTN, HLD, h/o PE, morbid obesity, recurrent falls, and chronic weakness with baseline limited mobility requiring a walker admitted for failure to thrive, AKI, hyperkalemia, and hyperglycemia.

#Fall
#Weakness
Pt with extensive comorbidities and chronic weakness. Acute on chronic exacerbation likely multifactorial and includes UTI, hyperglycemia, dehydration/AKI, deconditioning. Pan scans were negative, low suspicion for fracture or stroke given his exam and work up. Pt with hx of chronic LE weakness worse on the left side. Low suspicion for syncope given his history. Potassium slightly elevated w/o ECG changes, hyperglycemic to the 500s initially but improved to the 300s with insulin and fluids. No other metabolic abnormalities. Will continue to monitor and treat any underlying metabolic/infectious cause, pend PT/OT eval.

Plan:

0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**				
MRN:	3728595000001			Admit:	4/11/2026 21:10 PDT
FIN:	201841085			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

- Admit to MSU
- Vitals: q4hr
- Daily CBC, BMP, Mg, Phos
- Fall precautions
- PT/OT evaluation
- Monitor electrolytes and glucose closely
- Treat any underlying infection or metabolic abnormalities

#Hyperglycemia
Glucose in the 500s initially, repeat in the low 300s after fluids an insulin in the ED. Pt w/o anion gap, ketones. VBG was WNL. Low suspicion for DKA/HHS given his work up. Pt with no hx of DM. However, A1C has been increasing over the last year to as high as 6.8 in November 2025. Pt insulin naïve, will plan to initiate modest insulin regimen while inpatient. Calculated daily insulin with 0.3 units/kg/day.

Plan:
- IV insulin given (0.1 units/kg, 10 units) in ED
- Start 15 units Glargine nightly, 5 units of Aspart with meals
- Continue bedside glucose monitoring AC/HS
- Hold further fluid resuscitation give hx of HF
- Repeat HbA1c pending
- No evidence of DKA
- Cardiac diet

#Acute Kidney Injury
#CKD Stage III
Pt with baseline creatinine 1.8 to 2.1, now at 2.8 on admission. GFR is 23. Last RFP Nov 2025. Pt given 1 L of fluids in the ED.

Plan:
-Will hold further fluids for now given hx of HFrEF
-Hold home Bumex given new AKI
-Trend renal function daily
-Avoid nephrotoxins
-Renally dose medications.

#Urinary Tract Infection
Pt with reported frequency and dysuria. UA was infectious (nitrites, leukocyte esterase, >100 WBC, 4+ bacteria), likely contributing to the pt's increasing weakness. Pt received initial dose of Ceftriaxone in the ED. Will continue abx, f/u cultures.

Plan:
- Continue IV ceftriaxone, consider transition to oral abx if clinical picture improves
- F/u cultures
- Monitor for clinical response

#Non-Ischemic Cardiomyopathy s/p ICD
#HFrEF

0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**				
MRN:	3728595000001			Admit:	4/11/2026 21:10 PDT
FIN:	201841086			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

Last Echo July 2024 with EF of 50-55%, with aortic, mitral and tricuspid regurgitation. Pt with very mild LE edema which is now new per the patient. No other signs of volume overload on exam. BNP Mildly elevated. Delta troponins were negative. Will hold further fluids given hx.

Plan:
- Continue metoprolol succinate 50 mg daily
- Hold bumetanide 2 mg PO daily given AKI
- Strict I/Os
- Consider obtaining Echo inpatient

#Constipation
Pt with several days hx of constipation, last BM 1-2 days ago. No other GI sx. Mild bilat lower quadrant tenderness on exam. Mild stool burden w/o signs of obstruction on imaging.

Plan:
- MiraLAX 17 g daily
- CTM

#Hyperkalemia
K 5.4 w/o ECG changes. Likely elevated given hx of CKD and current K supplementation.

Plan:
- Cardiac monitoring
- Continue Sub Q insulin
- Hold home potassium chloride 20 mEq daily
- Trend BMP

#Atrial Fibrillation / Flutter
Hx of AFIB on Eliquis.

Plan:
- Continue apixaban 5 mg PO BID
- Continue metoprolol succinate 50 mg PO daily
- Telemetry

#Elevated Troponin
High-sensitivity troponin 32.37 to 32.88. No CP or SOB. Low suspicion for ACS, PE or other acute cardiopulmonary pathology given his work up and exam.
- Continue telemetry

#Open Wound of Buttock
Several small open wounds to buttocks in intergluteal cleft. No purulence, erythema or signs of infection.
- Wound care consult
- Monitor for infection

#Left Inguinal Hernia
No CT evidence of incarceration or obstruction

0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**				
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FIN:	201841085			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

- Monitor for pain, obstruction symptoms
- Surgical consult if clinical change

#Hypertension
- Continue metoprolol succinate 50 mg PO daily
- Hold bumetanide 2 mg PO daily given AKI
- Monitor BP

#Hyperlipidemia
- Rosuvastatin renally dosed to 10 mg PO daily (home dose 20 mg)
- Pending lipid panel

#Benign Prostatic Hyperplasia
- Continue finasteride 5 mg PO daily

#GERD
- Continue omeprazole 40 mg PO daily

#Chronic Pain
- Continue acetaminophen 1000 mg PO TID PRN pain or fever
- Continue lidocaine 5% patch, 12 hours on / 12 hours off

F: PO
E: Replete PRN
N: Cardiac Diet
GI: Not Indicated
DVT: SCD , Home Eliquis
Isolation: none
Code: FULL CODE
Dispo: MSU, pending clinical improvement, PT/OT, AKI improvement

Dylan Jacobsen, DO, Capt, USAF, MC
Family Medicine Resident PGY-1
99 MDG, 99 HCOS
Mike O'Callaghan Military Medical Center
Nellis AFB, NV

Pt seen with resident and agree with documentation.
//SIGNED//
Christina Roldan, MD, Capt, USAF, MC
Family Medicine Resident PGY-2
99 MDG, 99 HCOS
Mike O'Callaghan Military Medical Center
Nellis AFB, NV

Unless you are the subject of the record, protections under 42 CFR Part 2 and/or 38 U.S.C. 7332 may apply.

0079A-Mike O'Callaghan Military Medical Center

Patient Name:	**Downs, Jimmy**				
MRN:	3728595000001			Admit:	4/11/2026 21:10 PDT
FIN:	201841085			Discharge:	
Sex/DOB/Age:	Male	1/30/1951	75 years	Admitting:	JACOBSEN, DYLAN, DO, Family Medicine, Resident
DOD ID (EDIPI):	1060909365			Veterans ID (ICN):	

History and Physical Reports

Electronically signed on: 04/12/2026 02:26 PDT

DYLAN JACOBSEN
DO, Family Medicine, Resident

Electronically signed on: 04/12/26 05:37 PDT

DYLAN JACOBSEN
DO, Family Medicine, Resident

Electronically signed on: 04/12/26 06:17 PDT

CHRISTINA R ROLDANPROVIDEL
MD, Family Medicine, Resident

CHRISTOPHER M COLVIN
DO, Family Medicine

Requested review by:

RAMSEY L GARDNER
MD, Internal Medicine

Requested review by:

CHRISTINA R ROLDANPROVIDEL
MD, Family Medicine, Resident

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